

Section 13

# Known Fatality Risk Conditions & Pattern Evidence

Version 2.0 — Preliminary Draft

Petitioner: Ashley Marie Meredith, MSN | Pro Se

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## SECTION 13 — RISK & OUTCOME FAILURE STRUCTURES

Petition to Audit: Iowa Department of Health and Human Services

Child Welfare Systems — Longitudinal Accountability Record

Version 2.3 | June 5, 2026

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### SECTION PURPOSE

This section performs two analytical functions. Section 13.1 (Attachment A30) presents a structured Fatality Risk Analysis — a formal mapping of how documented operational deficiencies created, escalated, and failed to interrupt risk trajectories in confirmed child fatality and near-fatality cases in Iowa. Section 13.2 (Section 13 — Fatality Risk Analysis) presents a placeholder for the Systemic Failure Matrix, which is fully developed and presented in Section 13 of this petition (this section), following the evidentiary record established in Sections 6–12.

Section 13.1 connects the operational and oversight deficiencies documented in Sections 6 through 12 to measurable harm outcomes — demonstrating that the failure patterns identified in federal CFSR data, independent evaluations, AOS audit findings, and Ombudsman investigations are not abstract systemic concerns. They are the documented antecedent conditions for outcomes that include child fatalities, near-fatalities, and sustained post-permanency harm.

Cross-reference: Section 12 (Ombudsman fatality investigations — primary case sources);

**Section 6 (CFSR longitudinal data — performance baseline); Section 8 (federal intervention history); Section 14 (active case-level exemplars); Section 19 (Systemic Failure Matrix — complete analytical synthesis).**

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### 13.1 ATTACHMENT A30 — FATALITY RISK ANALYSIS

Analytical Classification: Petitioner-Generated Synthesis | Cross-Referenced Primary

Source: Ombudsman Fatality Investigations (A27, A28, A29); CFSR Round Data (A1–A4); AOS Reports (Series); Operational Reports (A5–A9)  
Domains: A; B; C; E; F; G; H  
deficiency categories: F1; F2; F3; F6; F9; F12

### **13.1.1 Analytical Purpose and Scope**

The Fatality Risk Analysis does not introduce new evidentiary claims. It applies a structured risk escalation framework to the evidentiary record already documented in Sections 6 through 12, examining specifically how documented operational deficiencies function as risk escalation mechanisms in confirmed child fatality and near-fatality cases investigated by the Iowa Office of Citizens' Aide / Ombudsman.

Three primary case studies anchor this analysis: the death of Shelby Duis (January 4, 2000), the death of Natalie Finn (October 2016), and the death of Sabrina Ray (May 25, 2017). A fourth case — Malayia Knapp (December 2015) — is included as a documented near-fatality within the same temporal cluster, and as the most probable subject of the third Ombudsman investigation disclosed in September 2020 but not published as of the date of this petition.

The analytical framework identifies three sequential risk escalation phases present across each case: (1) Risk Signal Generation — the production of observable, documented indicators of escalating harm; (2) Intake Suppression — the structural mechanisms that prevented those signals from entering the official response system; and (3) Intervention Failure — the breakdown of response mechanisms that, even when activated, failed to interrupt the escalation trajectory before irreversible harm occurred.

### **13.1.2 Risk Phase One — Risk Signal Generation**

In each of the four cases analyzed, the risk signal generation phase produced multiple, documented, actionable indicators of escalating child harm — indicators that were observable, reported, and in several cases legally mandated for investigation.

#### **SHELBY DUIS (2000):**

Between 1990 and 2000, three separate abuse reports were filed regarding the Duis household. The Ombudsman's investigation confirmed that all three reports should have been accepted for investigation under applicable Iowa standards. The final report was rejected three weeks before Shelby died. Hotline documentation of a prior call was not provided to the intake worker who made the final rejection determination — meaning the

worker who rejected the last report did so without access to the prior call record that would have materially informed that decision.

Risk signal profile: 3 abuse reports across 9 years; observable physical indicators documented in the case record; prior hotline contact history not integrated into intake decision-making. Risk escalation duration: 9 years of signal generation without system interruption.

### **NATALIE FINN (2016):**

Between January 2015 and October 2016, ten separate DHS reports were filed regarding the Finn household. Eighty percent were rejected at intake. The most critical intake rejection — for a report describing a "starving" child — omitted the word "starving" from the intake record. A safety assessment was completed before the assigned worker had physically seen the child. The assigned supervisor lost track of the case for 78 days during an active investigation period.

Risk signal profile: 10 reports over 21 months; word "starving" suppressed at intake; safety assessment completed before in-person contact; 78-day supervisory gap. Risk escalation duration: 21 months of escalating signals, 80% of which were rejected before reaching investigation status.

### **SABRINA RAY (2017):**

Between 2008 and 2017, eleven separate DHS reports were filed regarding the Ray household. One hundred percent were rejected or otherwise not substantiated. During the same period, Iowa DHS workers made regular visits to the Ray household — including during the period of Sabrina's active abuse — and documented suspicions without translating those suspicions into protective action. A licensed daycare contractor operating in the same household was actively coached to avoid triggering mandatory reporting obligations under Iowa Code § 232.69. A foster license was placed on administrative hold while children remained in the home. The household received \$640,000 in public subsidies during the documented abuse period.

Risk signal profile: 11 reports across 9 years; 100% rejection rate; regular DHS worker presence during abuse period; contractor coached to suppress mandatory reporting; administrative hold without protective resolution; \$640,000 in public subsidy payments continuing throughout. Risk escalation duration: 9 years of active and passive suppression of an escalating risk record.

### **MALAYIA KNAPP (2015):**

Malayia Knapp — an adopted child in the Urbandale household of Mindy and Anthony Knapp — was subjected to documented physical abuse, starvation, and confinement over an extended period prior to December 1, 2015. On December 1, 2015, Malayia fled the household. Two younger siblings were subsequently removed by DHS. Mindy Knapp was convicted of child abuse. Malayia filed a civil lawsuit against Iowa DHS in 2017 alleging that the post-permanency monitoring infrastructure failed to detect the ongoing abuse.

Risk signal profile: Adoptive placement with prior DHS contact history; post-permanency monitoring structure nominally in place; physical indicators of abuse, starvation, and confinement documented after flight. Risk escalation duration: Undetermined from public record; the outcome was survival only because Malayia was physically capable of self-escape at age 17.

### **13.1.3 Risk Phase Two — Intake Suppression Mechanisms**

Intake suppression — the structural failure to accept, process, or route risk signals into the investigative system — is the most consistently documented failure mechanism across all four cases. The Ombudsman's own comparative statement in the Natalie Finn report makes this convergence explicit: "Problems... in 2000 Shelby Duis report repeated in Natalie's case." (Natalie Finn Report, p.1)

This petition identifies six discrete intake suppression mechanisms documented across the four cases and the broader evidentiary record:

### **MECHANISM 1 — LEXICAL SUPPRESSION**

The omission of the word "starving" from the Natalie Finn intake record is a documented instance of lexical suppression — the alteration of a reported indicator at the point of transcription in a manner that reduces the apparent severity of the reported concern and thereby affects the intake determination outcome. This is not characterized as an isolated transcription error. It is a structurally significant failure because the word itself — "starving" — is the precise indicator that, if retained, would most directly require acceptance for investigation under Iowa intake standards.

### **MECHANISM 2 — HOTLINE RECORD NON-INTEGRATION**

In the Shelby Duis case, the hotline documentation of prior contact was not provided to the intake worker responsible for the final intake determination. The worker who rejected the last report before Shelby's death made that determination without access to the prior call record that would have established a documented pattern of household contact. The

CFSR Round 3 Statewide Assessment (2018) continued to identify systemic record integration failures as a root cause concern — 16 years after the Shelby Duis report first documented this mechanism.

### **MECHANISM 3 — MANDATORY REPORTER COACHING**

In the Sabrina Ray case, a licensed daycare contractor operating within the Ray household was actively coached to structure observations and communications in a manner designed to avoid triggering mandatory reporting obligations under Iowa Code § 232.69. This is the most serious intake suppression mechanism in this analysis because it is not a passive structural failure — it is an active institutional intervention to prevent the mandatory reporting system from functioning as designed. The identity and institutional affiliation of the individual who provided this coaching is documented in the Ombudsman's investigative report (A29).

### **MECHANISM 4 — NON-INTAKE STAFF SCREENING**

In the Shelby Duis case, the Ombudsman found that non-intake staff were screening hotline calls — making preliminary determinations about whether calls should reach qualified intake workers. This structural gatekeeping function, performed by staff who lacked the authority and training to make intake determinations, resulted in reports being filtered out of the system before reaching a qualified decision-maker. Iowa's 24% intake staff training compliance rate, documented in Ombudsman annual reporting (A26), establishes that this is not a historical anomaly confined to the year 2000.

### **MECHANISM 5 — PHYSICAL PRESENCE WITHOUT PROTECTIVE ACTION**

In the Sabrina Ray case, DHS workers documented suspicions during regular visits to the Ray household but did not translate those documented suspicions into protective action. The presence of documented suspicion in the case record, without corresponding protective intervention, represents a form of intake suppression that occurs not at the hotline but at the field level — where a worker's own documented observations are insufficient to trigger the response mechanism the system nominally provides.

### **MECHANISM 6 — ADMINISTRATIVE STATUS PARALYSIS**

In the Sabrina Ray case, the Ray household foster license was placed on administrative hold. Children remained in the household while the license was in administrative suspension. The administrative hold — which should have triggered either immediate protective removal or immediate administrative resolution — instead produced a documented

period of indeterminate status during which no protective action was taken and no resolution was reached. The mechanism designed to create protective intervention became the mechanism that deferred it.

#### **13.1.4 Risk Phase Three — Intervention Failure**

In the cases where reports were accepted for investigation, or where DHS workers had active case involvement, the intervention mechanism itself failed to interrupt the escalation trajectory before irreversible harm occurred.

#### **SUPERVISORY CAPACITY COLLAPSE:**

In the Natalie Finn case, the assigned supervisor lost track of an active investigation for 78 days. This is not documented in the Ombudsman's report as an individual personnel failure in isolation. It is documented as a structural product of supervisory overload, staff turnover, and the absence of case management systems sufficient to maintain supervisory awareness of active cases under investigation. The CFSR Round 3 Statewide Assessment (2018) identified caseworker and supervisor staffing as a primary root cause concern. The CWPPG (2017) documented crisis-level caseloads systemically. The CIA Report (2023) documented that the same supervisory capacity concerns remained structurally unresolved six years after the Natalie Finn case.

#### **SAFETY ASSESSMENT PROTOCOL FAILURE:**

In the Natalie Finn case, the safety assessment was completed before the assigned worker had physically seen the child. Iowa's Structured Decision Making (SDM) safety assessment protocol requires in-person contact as the foundational basis of the assessment.

Completing a safety assessment — a document that formally determines whether a child is safe in their current environment — without having seen the child renders the assessment structurally incompetent regardless of the content of the form itself. The CFSR SDM tool contract, which was a Round 3 PIP commitment, was not executed until April 2020 — after both the Natalie Finn and Sabrina Ray fatalities had already occurred.

#### **POST-PERMANENCY MONITORING ABSENCE:**

In both the Sabrina Ray and Malaysia Knapp cases, the fatal or near-fatal abuse occurred in adoptive or post-permanency placements. In the Sabrina Ray case, the same household that Iowa DHS had licensed as a foster home for nearly a decade — and compensated with \$640,000 in public subsidies — became the adoptive household where Sabrina died. In the Malaysia Knapp case, the post-permanency monitoring infrastructure failed to detect

years of documented physical abuse and starvation in an adoptive household until Malaysia was physically capable of self-escape.

The post-permanency monitoring gap is not a peripheral finding in this petition. It is a structural feature of the Iowa child welfare oversight system as currently designed: once permanency is achieved, the monitoring infrastructure that exists during foster care — FSRP services, ongoing-services caseworkers, licensing inspections, field worker visits — is substantially reduced or eliminated. This structural withdrawal of oversight at the point of permanency is the mechanism by which the post-permanency blind spot operates. It is documented in federal CFSR data, Ombudsman fatality investigations, and the active case record presented in Section 14 of this petition.

### **13.1.5 The Four-Case Convergence Pattern — Post-Permanency Risk Convergence Profile**

Across all four cases analyzed, the following five convergent failure elements are consistently present:

- (1) PRIOR DHS CONTACT HISTORY — All four households had documented prior DHS contact before the escalating harm event. In no case did the prior contact history generate a protective response sufficient to prevent subsequent escalation.
- (2) STATE-SUPERVISED OR STATE-DETERMINED PLACEMENT — All four children were in placements that were either actively supervised by Iowa DHS or had been established through Iowa DHS permanency determinations at the time the escalating harm occurred.
- (3) MANDATORY REPORTING SYSTEM FAILURE — In all four cases, the mandatory reporting system either failed to generate an accepted report, suppressed an existing report, or generated reports that were not translated into protective action.
- (4) POST-PERMANENCY OR LONG-TERM PLACEMENT STABILITY CONTEXT — In three of the four cases (Ray, Knapp, and the Drew household documented in Section 14), the escalating harm occurred after a permanency outcome had been established or after the placement had acquired long-term stability status within the DHS system.
- (5) DOCUMENTED STATE AWARENESS WITHOUT PROTECTIVE ACTION — In all four cases, the evidentiary record includes documentation of state awareness of risk indicators — through hotline calls, worker visits, case records, or licensing activity — without corresponding protective intervention sufficient to interrupt the escalation.

This five-element convergence pattern — Prior Contact + State Placement + Reporting System Failure + Post-Permanency Context + Documented Awareness Without Action —

is designated in this petition as the Post-Permanency Risk Convergence Profile (PPRCP). The PPRCP is not a retrospective analytical construct applied after the fact. It is a documented, observable pattern that was present in the evidentiary record of each case before the harm escalated to fatality or near-fatality status.

The PPRCP is also not limited to the four historical cases analyzed here. As documented in Section 14 of this petition, the same five-element pattern is present in the active case record involving the Drew household: prior DHS contact and documented concerns from 2018; state-determined foster placement beginning July 2018; repeated safety reports through multiple official channels; post-permanency context following the December 2019 TPR finalization; and documented state awareness across seven years without protective action. Section 14 does not present this as a litigation claim. It presents it as a contemporaneous indicator that the PPRCP pattern remains active within the Iowa child welfare system as of the date of this petition.

### **13.1.6 Audit Relevance of the Fatality Risk Analysis**

The Iowa Auditor of State is uniquely positioned to determine whether Iowa DHHS has conducted any internal analysis of the Post-Permanency Risk Convergence Profile — whether the agency has identified, studied, or implemented structural responses to the convergent risk conditions present across all four cases examined in this section.

Three specific audit questions are generated by the Fatality Risk Analysis:

AUDIT QUESTION 1: Has Iowa DHHS conducted any internal root cause analysis of the intake suppression mechanisms documented in the Shelby Duis (2000), Natalie Finn (2016), and Sabrina Ray (2017) investigations — specifically the lexical suppression, hotline record non-integration, and mandatory reporter coaching mechanisms — and if so, what structural changes were implemented as a result?

AUDIT QUESTION 2: Has Iowa DHHS implemented any post-permanency monitoring protocol or structural safeguard designed to address the documented monitoring gap that was present in the Sabrina Ray and Malayia Knapp cases — and if so, does that protocol apply to placements established prior to its implementation, including households such as the Drew household that were licensed under the prior monitoring structure?

AUDIT QUESTION 3: What is Iowa DHHS's current intake staff training compliance rate, and what structural mechanisms exist to ensure that the non-intake screening failure documented in the Shelby Duis case — and the 24% training compliance rate documented in Ombudsman annual reporting — have been structurally addressed rather than



procedurally noted?

The absence of documented structural responses to any of these three audit questions — or the presence of responses that were documented but not operationalized — is itself a significant audit finding within the scope of this petition.

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### **13.2 SYSTEMIC FAILURE MATRIX**

(Petitioner-Synthesized Body Content — Section 13)

#### **[PLACEHOLDER — FULL DEVELOPMENT IN SECTION 19]**

The Systemic Failure Matrix is the central structural pillar of this petition. It maps all documented deficiency findings across Sections 6 through 18 to the thirteen failure categories (F1–F13) and eight risk domains (A–H) defined in Section 5, producing a single consolidated analytical instrument that demonstrates the convergent, persistent, and institutionally embedded nature of the failure architecture documented throughout this petition.

The Matrix is developed in full in Section 19 of this petition, following the completion of all evidentiary sections (Sections 6–18), to ensure that it reflects the complete evidentiary record rather than a partial synthesis. Section 19 presents the Matrix in its final form alongside the analytical interpretation of convergence patterns, the self-reinforcing failure architecture finding, and the formal audit scope implications derived from the complete Matrix.

Cross-reference: Section 13 — Systemic Failure Matrix (full development).

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## **SECTION 13 — CROSS-REFERENCES**

Section 6: CFSR Longitudinal Data — performance baseline for risk escalation context

**Section 7: CFSR Round 4 Structural Integrity — current cycle convergence**

**Section 8: Federal Intervention & Judicial History — external validation**

Section 9: State Operational Self-Reporting — state admissions corroborating PPRCP

**Section 10: External Evaluations — independent corroboration of workforce and supervisory failure findings**

**Section 11: AOS Financial Oversight — concurrent fiscal monitoring failure**

**Section 12: Ombudsman & Complaint Systems — primary fatality case sources**

Section 14: Case-Level Outcomes — active confirmation of PPRCP pattern persistence

## Section 13:

Systemic Failure Matrix — complete analytical synthesis (this section)

## Section 19:

Corrective Action Matrix — remediation synthesis

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### 13.X DOMAIN & DEFICIENCY MAPPING — SECTION 13

Domain G — Federal Oversight History & Corrective Action Response (primary):

F5 — Corrective Action Failure (primary synthesizing deficiency):

### Section 13 is the convergence point for 23 years of documented

corrective action failure across all domains. Every F-category in

the deficiency framework is represented in the Section 13 matrix.

Domain H — Data Integrity & System Auditability:

F13 — Data Integrity & Reporting Failure: The PPRCP risk convergence

analysis depends on data reliability. The QA self-report discrepancy

pattern documented in Section 6 is a prerequisite risk factor for

all PPRCP findings.

All Domains A–H and all Deficiency Categories F1–F13 are mapped in the

Systemic Failure Matrix (Section 13 body). See matrix for full domain

and deficiency cross-reference by row.

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AI assistance disclosure: See Section 5.8.

primary sources cited in Sections 6–12 of this petition.

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End of Section 13 — Version 1.0 — June 5, 2026

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*AI assistance disclosure: See Section 5.8.*

*AI assistance was used materially in the preparation of this document. That assistance does not constitute independent verification of any factual claim.*